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Steven Posnack

Principal Deputy Assistant Secretary for Technology Policy

Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP)

Office of the National Coordinator for Health Information Technology (ONC)

Department of Health and Human Services

Mary E. Switzer Building,

Mail Stop: 7033A

330 C Street SW

Washington, DC 20201

Atten: HHS Health Sector AI RFI

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Mr. Posnack:

The Orthotic and Prosthetic (“O&P”) Alliance appreciates the opportunity to provide comments in response to the Department of Health and Human Services’ (“HHS”) Request for Information on the regulation, reimbursement, and research of artificial intelligence (“AI”) in clinical care. The O&P Alliance is a coalition of four major national O&P organizations representing over 11,000 certified O&P professionals and over 3,000 accredited O&P facilities that serve millions of individuals with disabling conditions in need of orthotic braces and prosthetic limbs. The O&P Alliance is committed to ensuring that individuals with limb loss and limb impairment receive timely and medically necessary O&P care from qualified practitioners and suppliers.

AI has the potential to improve patient outcomes, expand access to care, and reduce administrative inefficiencies. However, to realize these benefits, HHS will need to modernize reimbursement and other regulatory frameworks to enable the adoption of clinically appropriate AI-tools in O&P care. Also, HHS must also ensure that robust safeguards are in place to prevent bias in AI systems and prevent the misuse of AI by payers that could restrict access to care or erode patient protections, with little or no input from real clinicians who treat patients on a daily basis. This dual approach would help ensure that AI-tools are used to enhance, rather than replace, clinical judgement and patient-centered care.

HHS Must Adopt a Clinically Appropriate Framework to Support AI Use in O&P Care

AI holds significant potential for advancing O&P care across the full continuum of clinical services, including pressure mapping, gait analysis, and remote monitoring for long-term management of patients using prosthetic limbs and orthotic braces. AI-enabled tools can also aid orthotists and prosthetists in selecting the most appropriate orthosis or prosthesis and reducing the risk of adverse events (e.g., falls and skin breakdown). When appropriately implemented, AI-enabled tools can materially improve patient outcomes. In addition, AI-powered tools can help capture clinician-patient conversations, assist with documentation, and help reduce administrative burden, which allows orthotists and prosthetists to devote more time to direct patient care.

Given these benefits, it is critical that HHS adopt a regulatory framework that appropriately recognizes AI as a potential component of O&P care. Current Medicare and Medicaid regulations, however, do not adequately account for AI-enabled software and tools. For instance, there are no Level II Healthcare Common Procedure Coding System (“HCPCS”) codes that specifically describe or value AI-embedded software or tools in O&P technologies. As a result, manufacturers who invest in these technologies often cannot achieve the necessary return on investment due to the lack of reimbursement from Medicare or Medicaid, despite these tools demonstrably improving patient care. This misalignment between technological advancements and payment policy creates a barrier to innovation and adoption within the O&P profession, and limits patient access to clinically beneficial technologies.

Therefore, we urge HHS to address the absence of clear coverage/reimbursement pathways for AI-enabled tools integrated into O&P devices and technologies. This pathway should address, for instance:

- Embedded software within orthoses or prostheses;
- AI-driven analytics used for clinical assessment, fitting, and tracking for patients in need of orthoses or prostheses;
- Ambient listening and documentation tools powered by AI to improve the clinician/patient encounter and clinical workflow; and
- Remote monitoring platforms that support ongoing management of O&P care.

Modernizing regulatory frameworks may require the creation of new HCPCS codes and reimbursement levels, revisions to established coverage and coding guidance, and alternative payment mechanisms that recognize the value of AI-enabled tools in O&P care. Where statutory constraints limit such reforms, HHS should work with Congress to obtain the necessary authority.

HHS Must Implement Guardrails on the Use of AI for Claims Processing and Prior Authorization Requests

Although AI may serve as a valuable clinical tool when used to assist clinicians in medical decision-making, we are concerned that some payers are deploying AI-enabled systems to

automate claims/prior authorization denials or otherwise restrict access to care without meaningful human review or individualized, clinically-based determinations of medical necessity. In the context of O&P care, where treatment decisions are highly patient-specific, such practices by payers may result in inappropriate denials or delays in care, leading to preventable health deterioration, loss of mobility, and loss of independence. It may also force orthotists and prosthetists to pursue lengthy and costly appeals to reverse the payer's improper claim or prior authorization denials. AI should not be permitted to function as a substitute for clinical review nor should it be used by payers to circumvent existing coverage or reimbursement standards at the expense of practitioners and patients.

Therefore, HHS should ensure that payer use of AI in claims adjudication and utilization management remains subject to robust transparency and meaningful human oversight, including:

- Requiring that a clinician with relevant qualifications review all denials of claims and prior authorization requests;
- Requiring payers to clearly inform patients and practitioners when and how AI tools are used in claims processing or prior authorization decisions;
- Allowing practitioners and patients to opt-out of participation in any program that relies on AI-enabled tools to render coverage or payment determinations;
- Requiring payers to remain fully responsible for determinations made with AI-enabled tools and to explain the individualized basis for those decisions;
- Ensuring that patients and practitioners are able to confer with qualified clinicians and other payer employees with respect to questions regarding prior authorization requests and claim processing;
- Require payers to conduct internal audits to ensure that any AI-enabled tools comply with all applicable coverage standards; and
- Require HHS to monitor for discriminatory or biased outcomes that could disproportionately impact individuals with disabilities or other complex medical conditions.

HHS Must Develop Policies that Address the Bias

The use of AI raises concerns regarding algorithmic bias and its potential to adversely affect individuals with disabilities. Many AI tools rely on pattern recognition derived from large datasets that often inadequately represent individuals with disabilities, including individuals with limb loss and limb difference. Because O&P patients often present with complex, highly individualized medical conditions, AI systems trained on generalized datasets may not accurately capture their unique clinical and functional needs or outcomes. This skewed data can result in significant bias that adversely impacts individuals with disabilities. Bias can also occur when AI-tools rely on flawed proxies, such as healthcare costs, to aid in the clinical decision-making process. For O&P patients, these proxies can underestimate clinical care needs and the long-term value of certain treatments, which could result in inappropriate restrictions on care. Accordingly, overreliance of AI, without appropriate safeguards, may ultimately lead to inappropriate care, delayed care, or denied care.

To help address bias in AI systems, HHS should ensure that AI-enabled tools used by practitioners and payers are subject to robust safeguards, including:

- Rigorous testing for accuracy and bias specific to individuals with disabilities;
- Ongoing monitoring of AI to identify disparate impacts on individuals with disabilities; and
- Research to study prevalence of bias in AI-enabled tools used by practitioners and payers.

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To contact the O&P Alliance directly, please contact Peter Thomas, O&P Alliance Counsel, at 202-872-6730 or at Peter.Thomas@powerslaw.com, or Leela Baggett at Leela.Baggett@powerslaw.com.

Sincerely,



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